

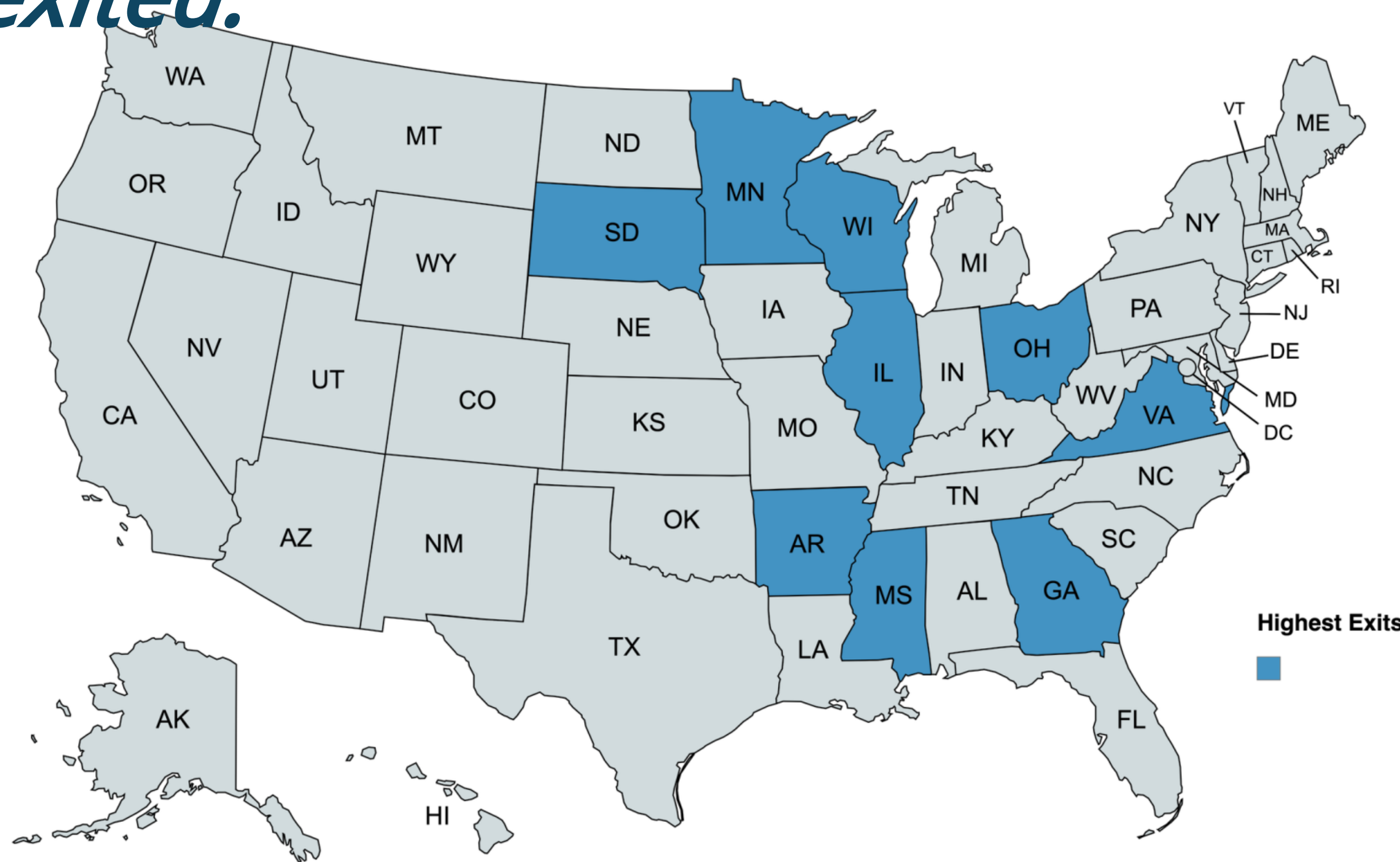


ACHP Medicare Advantage Market Exit Analysis



*43 states. 1,268 counties. 607 plans
exited.*

10 states with
highest exits



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Bottom Line Up Front

ACHP is the market backstop — capturing 3× more share where national carriers exited.

0.82%

Share gain in exit counties

vs. only 0.28% in stable markets — a 3× difference

Late 2025

When the gap appeared

ACHP share diverged sharply starting late 2025

MN & ND

Where gains concentrated

Strongest pickup in Midwest — sponsor's priority region



Three Recommendations

ACHP is the market backstop — here is how to use it.

01

Build the early-warning dashboard

Monitor star rating trends, HHI shifts, and carrier filing signals monthly. Prioritize counties with high PPO and HMO-POS concentration. Act within the 3-month pickup window — before displaced beneficiaries choose someone else.

Timing is everything

02

Deploy the county triage framework

Classify flagged counties into two action tracks:
Expand — concentration↑ & ACHP share↑ (185 counties): double down, scale what's working.
Defend — concentration↑ & ACHP share↓ (73 counties): intervene immediately.

Expand 185 | Defend 73

03

Pilot PPO/HMO-POS expansion in MN & ND

The Midwest playbook works. Minnesota and North Dakota show the strongest gains — ACHP's priority region. Scale the model that's already proven before the next wave of carrier exits hits.

Strongest pickup region

Roadmap



01

The Situation

MA market under pressure:
carrier exits & financial stress

02

The Problem

SCQ: do ACHP plans capture
displaced beneficiaries?

03

Our Approach

Data pipeline, EDA,
and analytical framework

04

Findings

Market share, timing,
drivers & geography

05

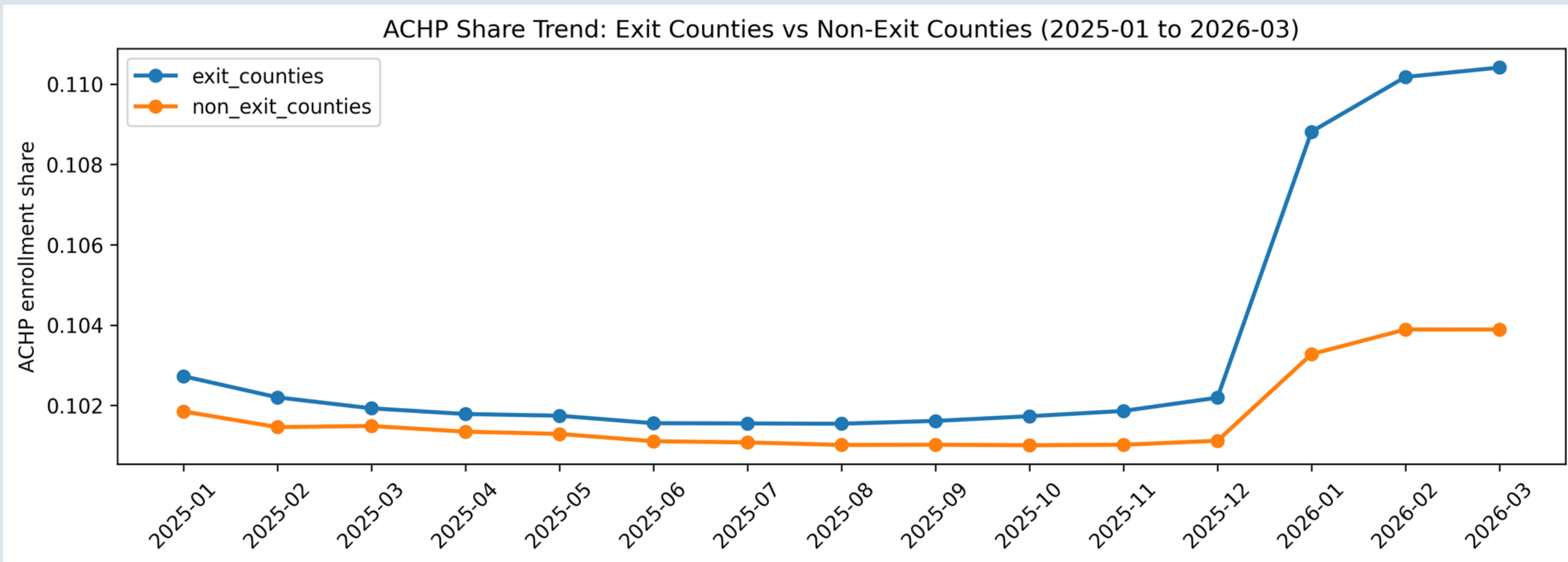
Recommendations

3 actionable strategies
for ACHP leadership



ACHP Share Trend (Exit vs Non-exit)

ACHP share increased more in exit counties than non-exit counties

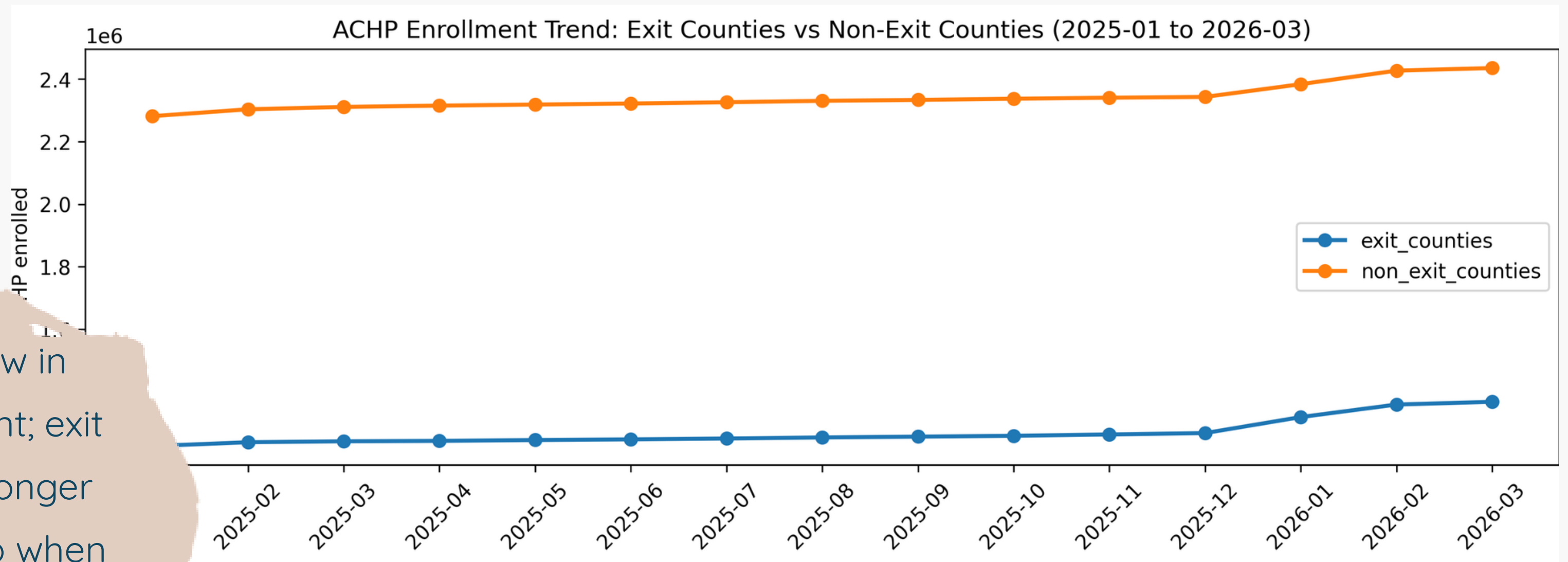


Share gains accelerate after late 2025 in exit counties, suggesting reallocation rather than baseline growth.

- Share = pooled ACHP enrolled / pooled total enrolled within each group (not simple county average).
- Source: CMS Monthly MA Enrollment (State/County/Contract) + ACHP crosswalk.

ACHP Enrollment Trend (Exit vs Non-exit)

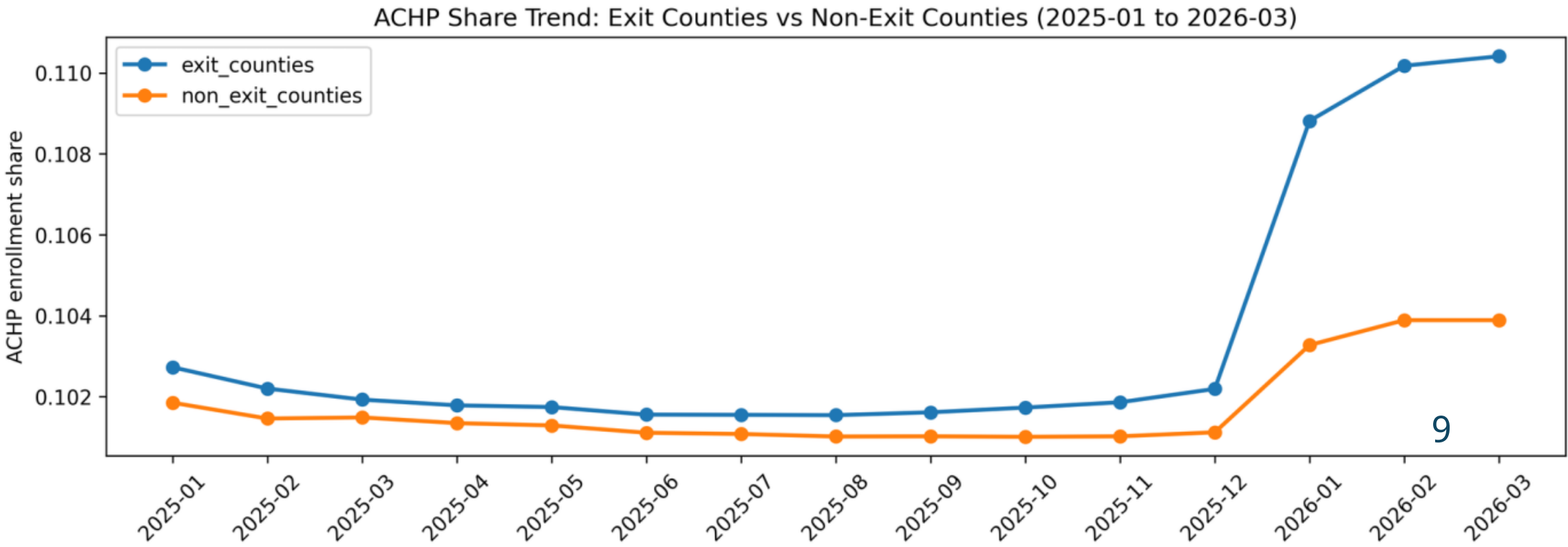
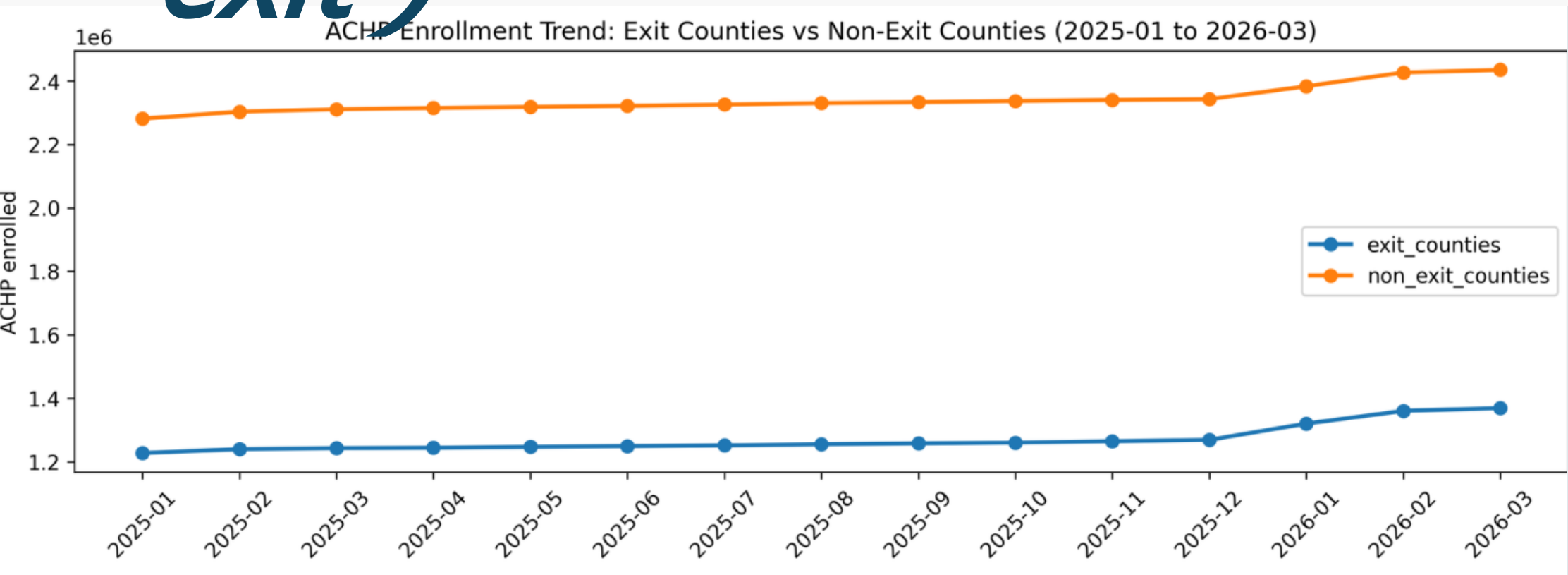
Enrollment grew broadly, but share gain is the differentiator



Both groups grew in absolute enrollment; exit counties show stronger competitive pickup when normalized by market size.

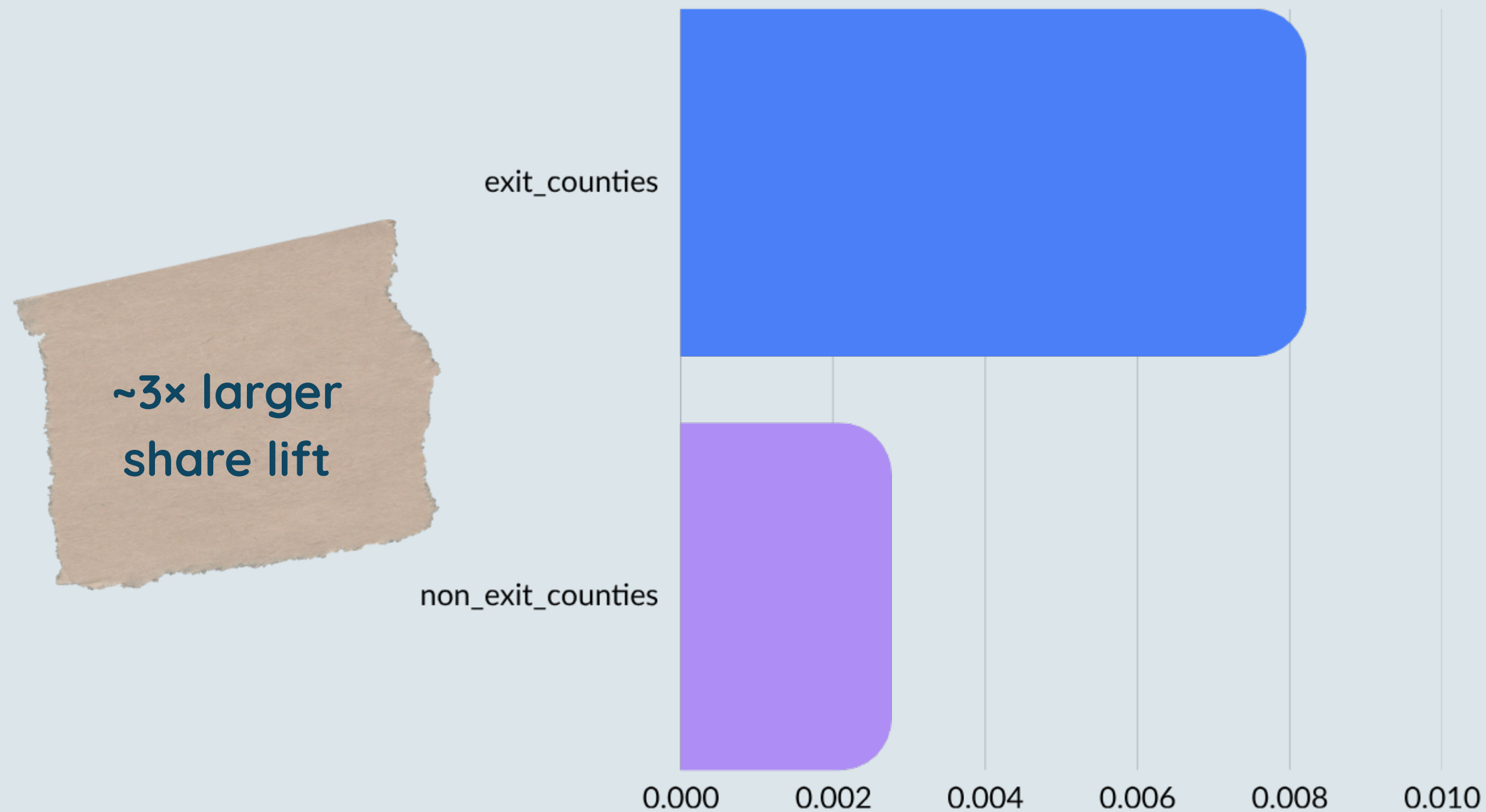
ACHP Share & Enrollment (Exit vs Non-exit)

why the share lift is much larger in exit counties?



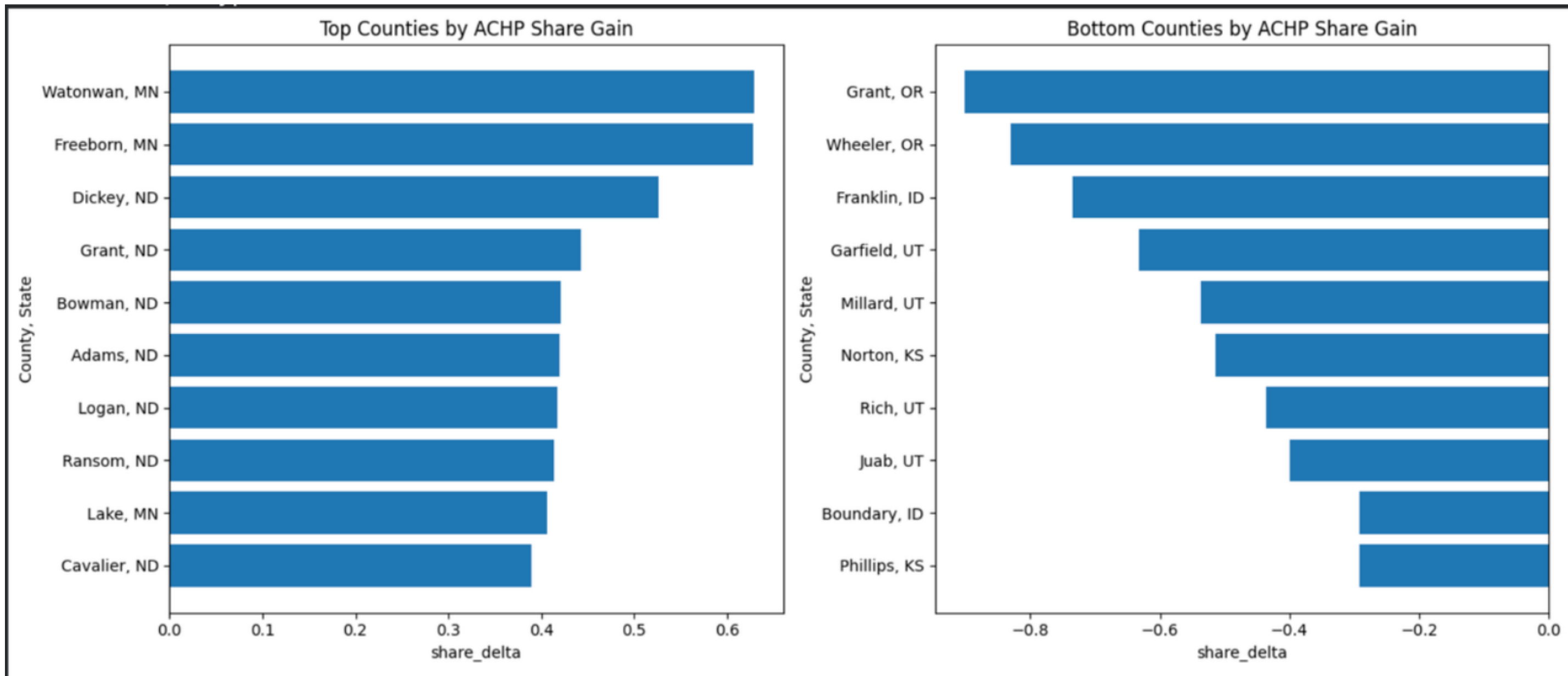
Data Analysis

Exit counties: +0.82pp vs Non-exit: +0.28pp (Dec'25 → Mar'26)



Next: Why do some exit counties win and others lose? → Triage

Carrier Exits Created Opportunity — But ACHP Only Captured It in Some Markets



Best: +63% share gain (Watowan, MN)

Worst: -82% share loss (Grant, OR)

Recommendation: Build a carrier exit early-warning system

■ *Hypothesis*

H1a: Counties with more carrier exits should show higher ACHP pickup gains.

x Near-zero correlation

H1b: Counties where exiting plan types better match ACHP offerings should show higher pickup

x Direction reversed

H1c: Counties where ACHP offers lower MOOP than exiting plans should show higher pickup

x Gap size does not predict pickup

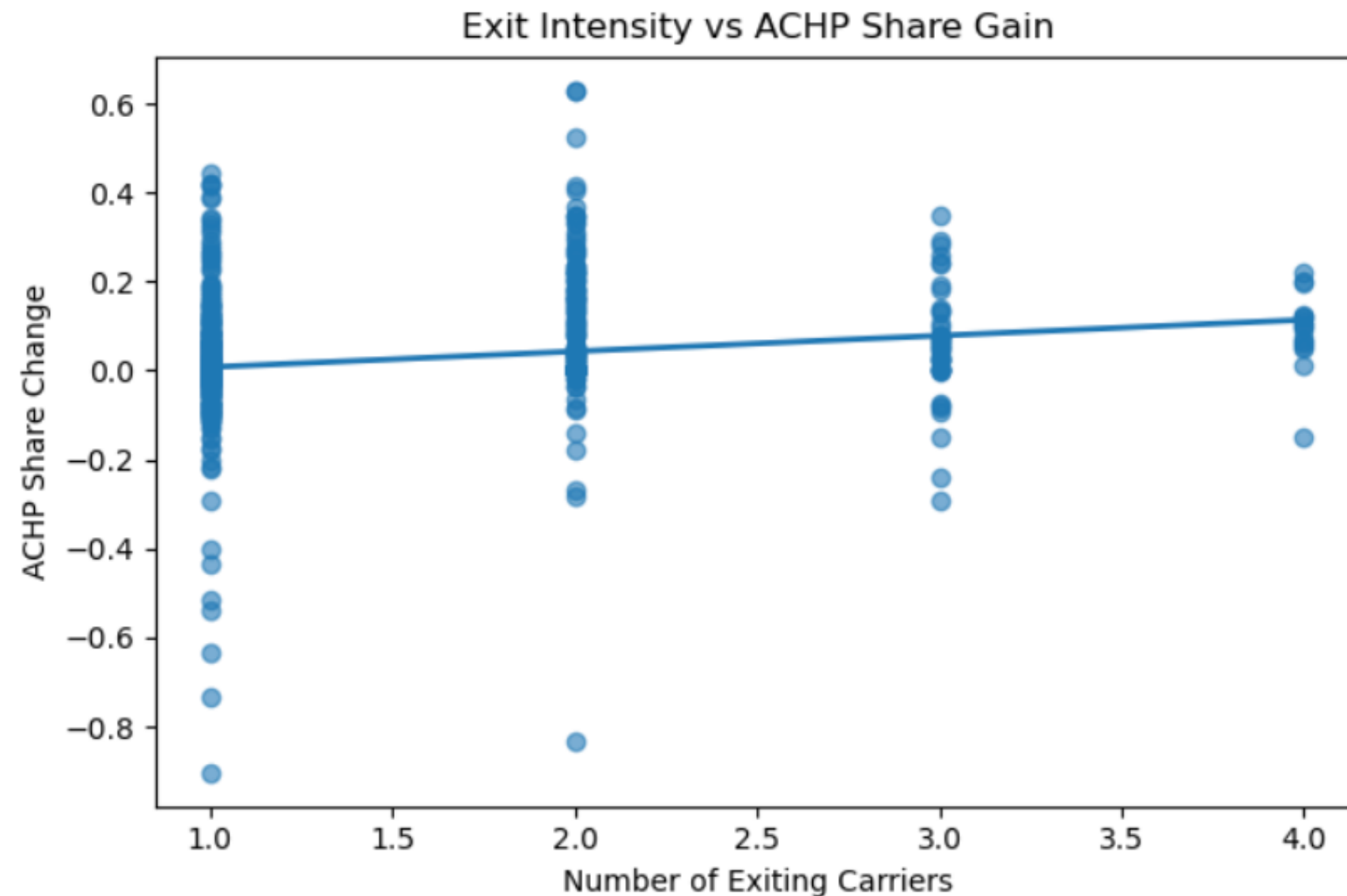
H2: Counties where stronger-quality plans exited should show higher market quality decline and higher ACHP pickup gains

✓ Supported, But as ACHP gains pickup, its own quality advantage is shrinking

H3: Market-structure changes in exit counties should create both opportunities and risks for ACHP.

✓ Supported: High- Δ HHI exit counties split into ACHP share-gain and share-loss markets, enabling Expand vs Defend triage.

H1a: More Carrier Exits Do Not Translate to Higher ACHP Pickup

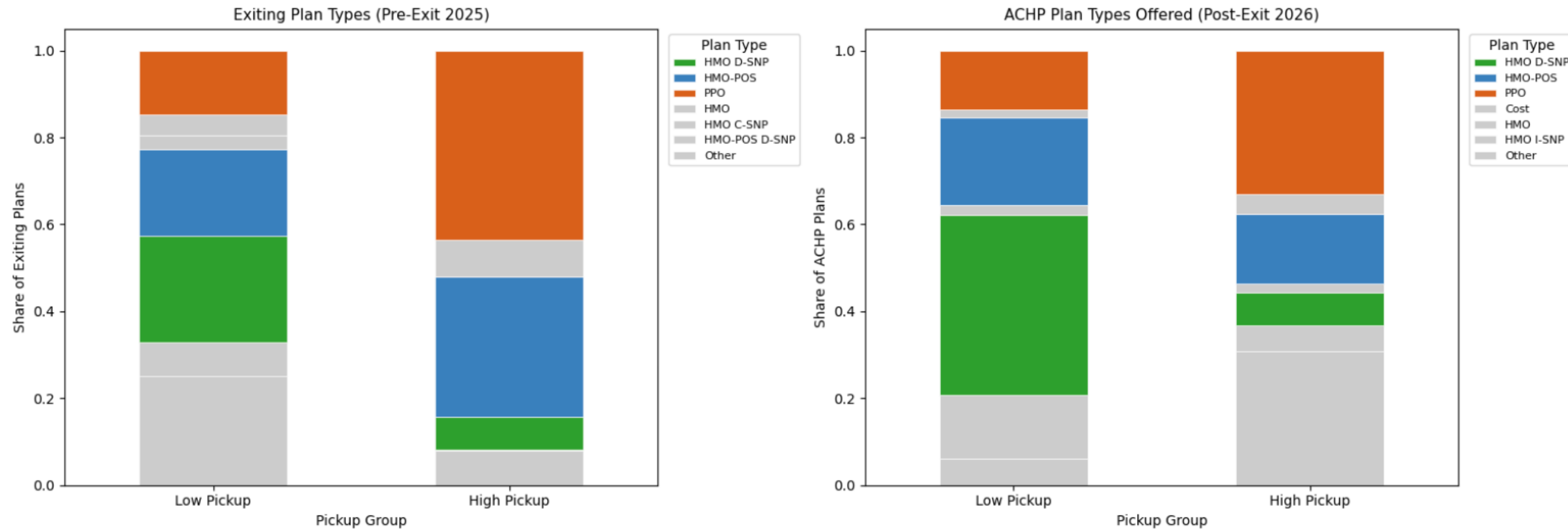


- Hypothesis: counties with more exits should show higher ACHP share gains
- Finding: near-zero correlation , the regression line is flat across 1–4 exiting carriers
- Exit volume is not a reliable predictor of ACHP opportunity

If volume doesn't explain it — does product type?

H1b: ACHP Gains Most Where Product Match Is Worst — Pointing to Market Vacuum

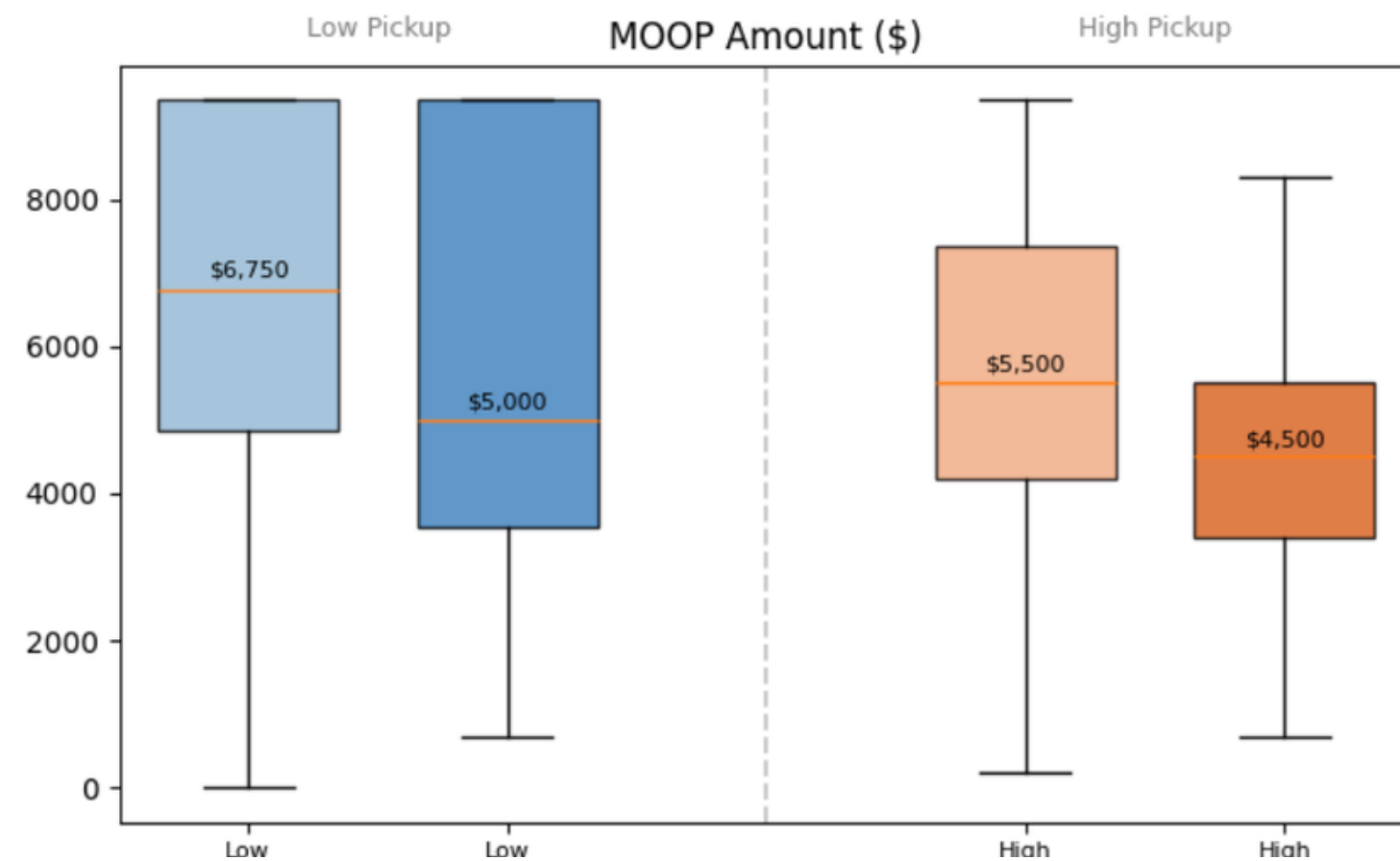
Analysis B: Plan Type Gap
(What exited in 2025 vs What ACHP offered in 2026)



- High Pickup: PPO and HMO-POS made up 76% of exits
ACHP did not fully replace them
- Low Pickup: D-SNP well matched — yet pickup stayed low.
Better match did not drive pickup

Where product fit is worst,
market vacuum is strongest ,
and ACHP gains most.

■ *H1c: ACHP's Structural MOOP Advantage Becomes a Conversion Asset When Competitive Vacuum Forms*

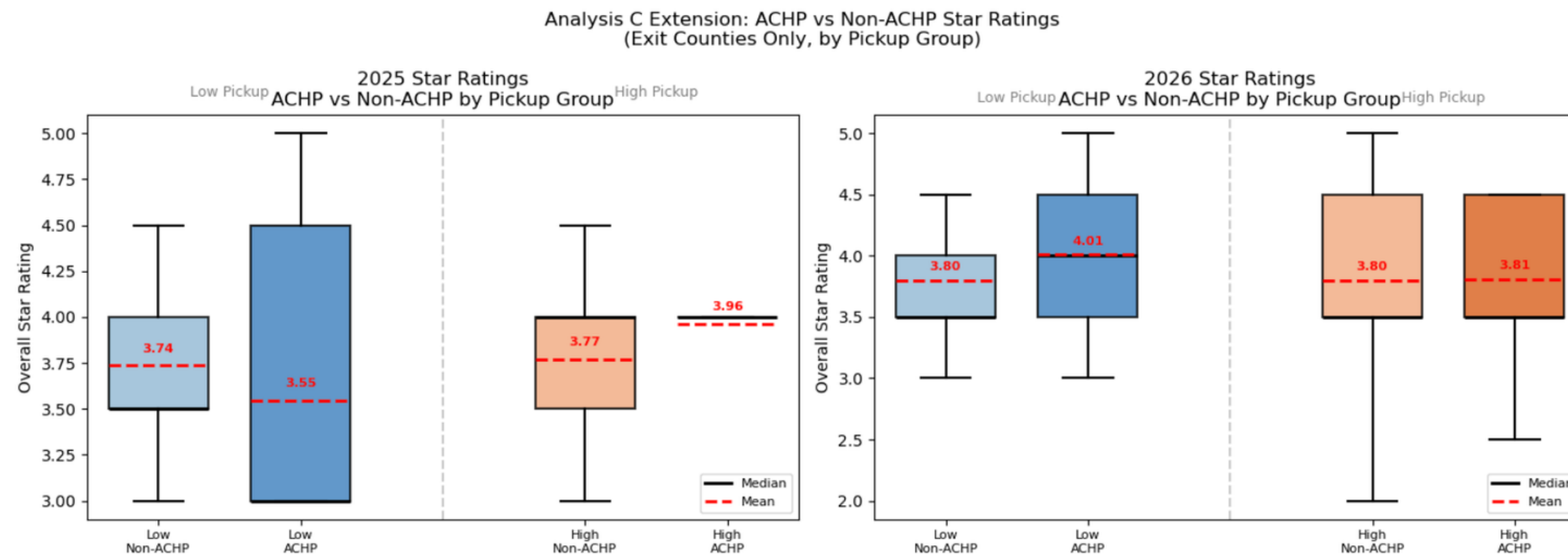


- ACHP MOOP consistently lower across all markets: \$1,750 gap in Low Pickup, \$1,000 in High Pickup
- Not a pickup driver
A conversion asset that activates when strong competitors exit and choices narrow

MOOP tells us why beneficiaries choose ACHP.

The early-warning system tells us where to be when they need to choose

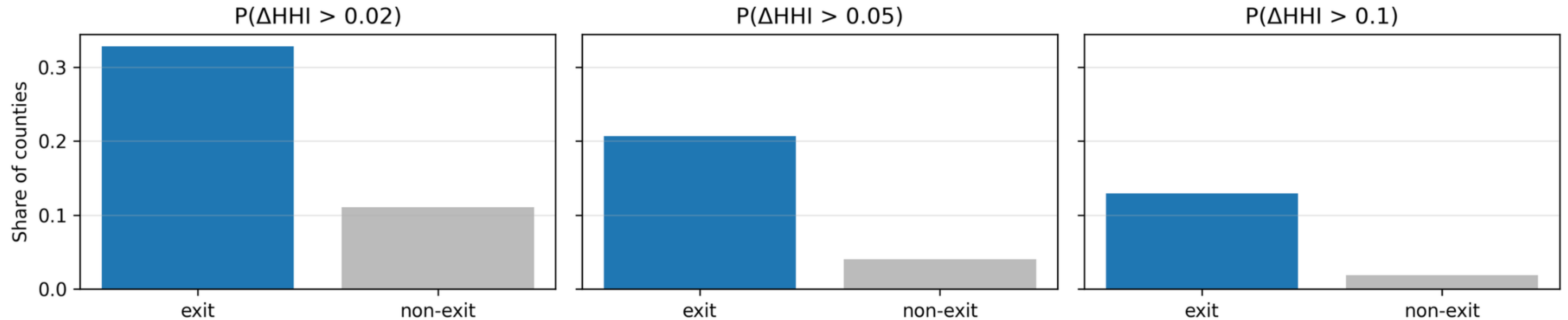
■ *H2: Where Strong Competitors Exit, Quality Falls and ACHP Gains Most*



Quality must be actively defended or pickup gains will reverse.

- 2025: ACHP star rating higher than Non-ACHP in High Pickup counties
- 2026: Gap nearly eliminated
- Low Pickup counties improved (+0.211) while High Pickup stagnated
rapid growth may be diluting quality

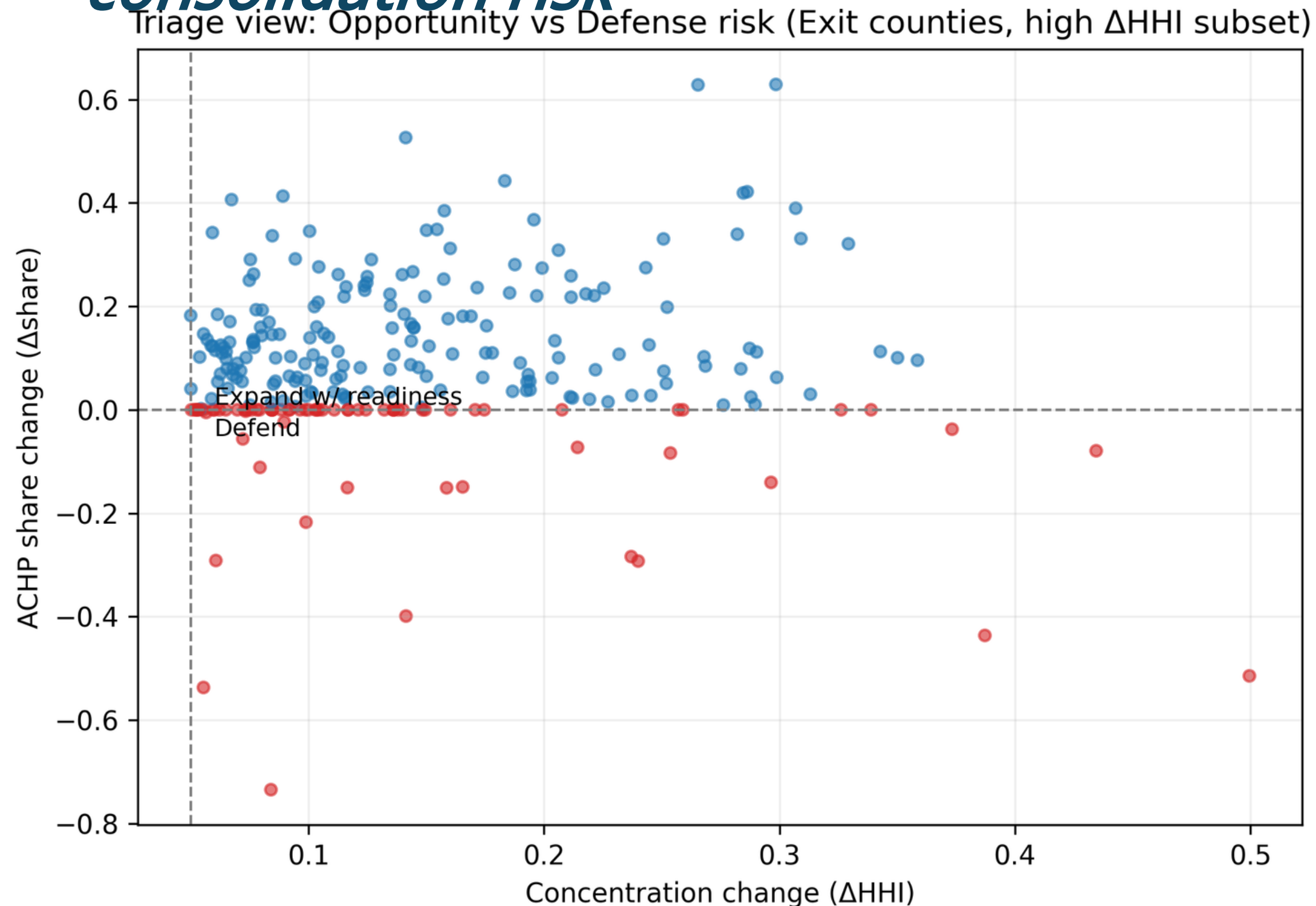
■ *H3: Market-structure changes in exit counties should create both opportunities and risks for ACHP*



- Concentration = enrollment consolidates around a few winners
- $\Delta HHI = HHI(2026-03) - HHI(2025-12)$

Exit counties are consistently more likely to show large ΔHHI

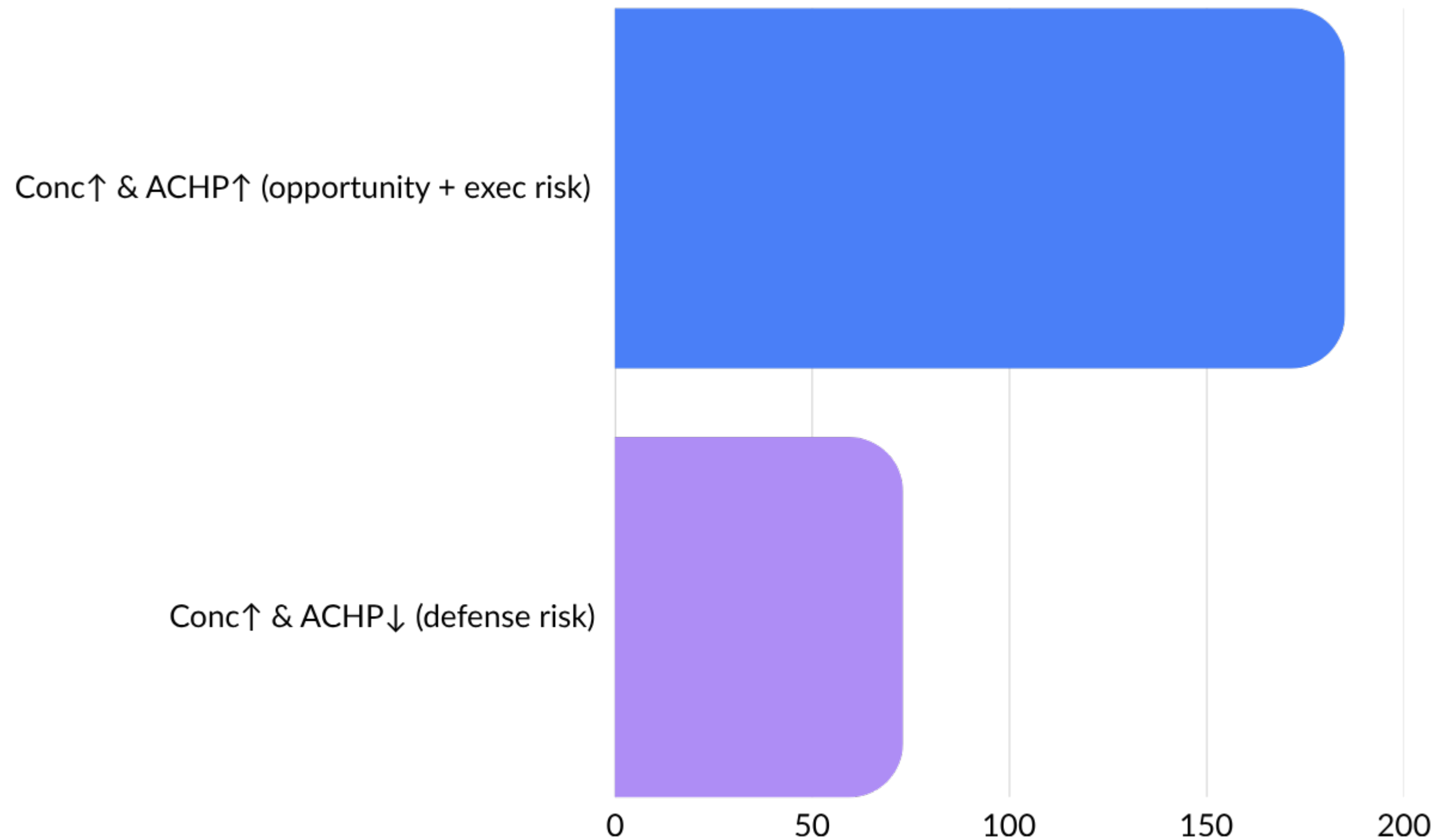
■ Evidence of consolidation risk



- Market concentration creates both upside and risk for ACHP.
- Share-gain counties = expand, but prepare capacity.
- Share-loss counties = defend, because competitors may be winning faster.

This indicates greater volatility and execution risk post-exit.

■ *Triage → action lists*



- Expand: Concentration ↑ + ACHP share ↑ (185 counties → scale with readiness)
- Defend: Concentration ↑ + ACHP share ↓ (73 counties → immediate risk response)

This segmentation directly informs the early-warning dashboard recommendation

Conclusion

ACHP is the Market Backstop — Let's Position Us to Embrace It

Carrier exits are the new normal

Disruption will keep creating openings

ACHP captured share where competitors fled

With no quality trade-off

Reinforcement of ACHP's mission

Serving beneficiaries when national carriers walk away

Preempt Competitor Exits

Deploy a monthly early-warning system

Safeguard Quality Leadership

Implement dual-path Star Rating monitoring

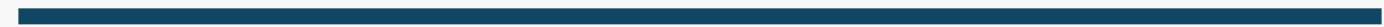
Operationalize Targeted Triage

Apply the "Expand vs. Defend" framework

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Q&A



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Appendix

	market_type	achp_2025_12	achp_2026_03	achp_delta	share_2025_12	share_2026_03	share_delta
0	exit_counties	1268468	1368492	100024	0.102191	0.110410	0.008219
1	non_exit_counties	2343051	2435246	92195	0.101118	0.103889	0.002771

Appendix

1.Data Sources and Exit Definition

Dataset	Source	
MA Landscape File	CMS	CY202
Monthly MA Enrollment	CMS SCC	2025-0
ACHP Member Crosswalk	ACHP	2025-1

2.High / Low Pickup Classification

Variable: share_delta = ACHP enrollment share (2026-03) – ACHP enrollment share (2025-12)

Classification:

High Pickup = share_delta ≥ Q75

Low Pickup = share_delta ≤ Q25

Middle group excluded from analysis

3.Quality Cost — Paired t-Test Results

Group	Pre-Exit	Post-Exit	Delta
All Exit Counties	3.748	3.79	0.042
High Pickup	3.811	3.77	-0.041
Low Pickup	3.726	3.796	0.07

t = 7.530, p = 0.0000 — statistically significant